

Provision of health services to nursing home residents: policy options for Europe

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List of abbreviations

ADL	Activities of daily living
CGA	Comprehensive geriatric assessment
ECP	Elderly care physician
GP	General practitioner
LTC	Long-term care
MeSH	Medical Subject Headings
NHS	National Health Service
WHO	World Health Organization

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Executive summary

The challenges around health service access for nursing home residents mirror wider issues across European health systems. A robust primary care foundation – delivering proactive, person-centred care and a broad range of preventive services – together with multidisciplinary teams that include allied health professionals is essential. This approach is particularly crucial for the chronic disease, multimorbidity, and complex geriatric profiles typical of nursing home residents. Ideally, primary care functions as the coordinating hub and gatekeeper, ensuring well-integrated specialist input and reducing fragmentation.

The provision of health services to nursing home residents is crucial to slow down functional decline, prevent avoidable hospitalizations and maintain or even improve functional ability and quality of life, thereby improving quality of care and health outcomes.

This report provides an overview of the provision of health services to nursing home residents in seven European countries: England (United Kingdom), Germany, the Netherlands,¹ Norway, Poland, Spain and Sweden. It aims to identify policy options for strengthening health service provision to nursing home residents.

The report finds that the provision of primary care, care by allied health professionals, and specialized care to nursing home residents varies substantially between and within the selected European countries.

In most of the seven countries, long-term care (LTC) services are covered and financed similarly to health services, but co-payments may play a substantially larger role in LTC spending than in health spending. The proportion of nursing homes operated by non-profit, for-profit, public and private providers differs between the selected countries: while Germany and the Netherlands have many private non-profit operators, most nursing homes in Norway, Poland and Sweden are public. In Spain and England the majority of nursing homes are privately owned and for-profit.

Medical care tends to be primarily provided by general practitioners (GPs), while nursing staff or care assistants provide 24-hour care. However, most GPs working in nursing homes are self-employed and community-based, and not employed by the facility. As a result, they only spend a certain amount of time per week with the nursing home residents, while working the rest of the time in their own practice.

An exception is the Netherlands, where specially trained elderly care physicians (ECPs) are directly employed by larger nursing homes, and some nursing homes in Norway follow a similar model. There is some evidence that this ECP model can help to reduce the demand for specialized services, avoid unnecessary hospitalizations and lead to higher patient satisfaction with primary care services.

Patients in general in all seven countries are free to choose their GP and this can affect the provision of primary care to nursing home residents. Countries differ in particular in terms of whether nursing home residents keep the GP they had prior to transitioning to the nursing home (assuming it is close to where they previously lived).

The payment schemes for physicians providing services to nursing home residents also differ, comprising capitation and fee-for-service models, as well as fixed salaries. In Germany, for example, physicians are reimbursed through a fee-for-service model, whereas in Norway they tend to be employed by the municipalities and receive a fixed, regulated salary. In Poland and Spain both

¹ Note that Netherlands (Kingdom of the) comprises six overseas countries and territories and the European mainland area. As data for this brief refer only to the European territory, the report refers to it as the Netherlands throughout.

models exist and GPs providing services to nursing home residents may be either self-employed or salaried employees.

In all seven countries many nursing homes involve physiotherapists, occupational therapists, speech and language therapists, psychologists and social workers. Allied health professionals are either directly employed by the nursing homes or work independently in private practices and provide services upon referral, either on-site or at their own practice.

Whether allied health professionals are directly employed by the nursing home or work on a freelance basis depends on the country, the type and ownership of the nursing home, the region in which it is located (rural vs. urban), its size, specialization and local policies. There are also differences between the different categories of allied health professionals.

The services of allied health professionals (whether inside or outside the nursing home) tend to be publicly covered, provided they are based on a referral from a GP.

Similar to allied health professionals, specialists are often brought in as consultants, rather than being part of the team working permanently in nursing homes. Specialists usually work in private practices and may offer nursing home visits.

Upon admission to a nursing home in all of the selected countries, a geriatric assessment is carried out to develop a care plan. This is usually carried out by a physician, but allied health professionals may also play a role.

Access to GPs and other health professionals is often organized by nursing staff, who have the closest contact with the nursing home residents. In most of the selected countries GPs act as “gatekeepers” to other medical services and their referral is needed for specialist appointments, for example in Poland and Spain. However, in some countries, such as Germany and England, nursing home residents can also choose to consult specialists independently.

Where feasible, primary, specialized and intermediate care should all be provided within nursing homes, minimizing care transitions and avoiding acute hospital admissions, disorientation and distress. The ideal is a proactive and person-centred approach that consistently meets high standards of care.

Many of the challenges highlighted in this report extend beyond nursing home residents and reflect broader health and LTC system questions: the development and gatekeeping role of primary care, the coordination of care across settings, and equitable access to publicly covered allied health services.

To date, many decisions on how to ensure access to primary care, allied health professionals, and specialized services for nursing home residents have been taken implicitly or by default. Greater transparency and explicit decision-making could already yield improvements, and this report hopes to provide pointers to underpin these efforts.

Key messages

- Nursing home residents need access to primary care, specialized services and services by allied health professionals. How this is provided and paid for varies across Europe.
- Providing proactive health care on site in nursing homes – including chronic disease management, prevention and health promotion – helps
 - residents preserve their mobility, health status and quality of life.
 - reduce avoidable hospital admissions, disorientation and distress.
- The way health services are provided in nursing homes depends on the specific health system and long-term care context (and can vary within countries). The role of primary care - its coordinating and gatekeeping functions and how it is delivered – is particularly important.
- Models of service provision need to reflect the surrounding healthcare infrastructure but also practical factors such as the size of the nursing home.
- Critical policy choices that shape continuity, coordination and quality of care include where health workers caring for nursing home residents are based, how they are paid and the range of services they provide.
- Different models of provision have different advantages and challenges:
 - Allowing residents to keep their 'old' GP supports relational continuity but can lead to infrequent visits, limited out-of-hours cover and leave homes dealing with multiple practitioners.
 - Cooperation agreements between nursing homes and a GP practice or primary healthcare centre can offer better coverage and continuity, but depend on there being an appropriate group practice nearby that can support backup and after-hours care.
 - GPs working as part of a multidisciplinary team can add value by supporting more comprehensive care delivery, improving coordination and contributing to quality.
 - Dedicated nursing home physicians is perhaps the optimal model and can offer high-quality GP care and better health outcomes, but is only tenable in larger homes. It may cost more initially but is likely to save on specialized services in the long run.
- The role of allied health professionals (and how far their services are publicly covered) is also context-dependent but they can play a crucial role in assessments, in recognizing specific medical needs and in offering appropriate care and rehabilitation.
- Policy-makers face choices on whether nursing home residents should access health services following the same pathways and referral mechanisms as the general population or through tailored or expedited arrangements reflecting their special needs.
- Policy changes could improve coordination and foster more equitable, better quality and more person-centred care. Changes might usefully include
 - Greater transparency and explicit decision-making on how nursing homes are integrated into the broader health and social care framework.
 - Robust quality assurance mechanisms and governance structures to offset the consequences of the very different approaches to organizing and financing care.
 - Strong mechanisms for integrating care and for managing the transition between care settings, including through GP gatekeeping.

The provision of primary and specialized health services to nursing home residents is crucial to slow down functional decline, avoid hospitalizations and maintain or even improve functional ability and quality of life, improving quality of care and health outcomes. This requires well developed primary care services and the possibility to refer people to specialists where needed (Brett et al., 2019; Sterke et al., 2021).

This report provides an overview of the provision of health services to nursing home residents in seven European countries: England, Germany, the Netherlands, Norway, Poland, Spain and Sweden. It aims to identify policy options for strengthening health service provision to nursing home residents.

Europe has one of the oldest populations in the world and, after the setback of the COVID-19 pandemic, the average European lifespan is increasing further, while birth rates remain low. As a result of these demographic developments and overall population ageing, the use of health services and LTC also increases, as people are more likely to have health or functional limitations and depend on care when they get older (Theisen, 2020).

As populations age and experience increasingly complex health needs, nursing home residents represent a growing and vulnerable group with a high level of frailty, comorbidity and cognitive deficits (Glette et al., 2018; Spreckelsen et al., 2020b; Honinx et al., 2021a; Fassmer et al., 2024). It is therefore crucial to understand existing care structures and to assess their preparedness for the growing demand for geriatric care and related LTC services (Grol et al., 2021; van de Haar & Alme, 2025).

Nursing homes in Europe

In light of this growing demand for care, the provision of LTC is becoming ever more important. One approach is to provide LTC in residential facilities, such as nursing homes, where people who are no longer able to live independently can receive full-time residential care 24-hours a day (Sattar et al., 2023). In addition to residential and nursing care, such facilities may offer primary and specialized care, although the organization and provision of health services in nursing homes varies markedly between European countries (Sattar et al., 2023). Primary care refers to those services provided by first line health and social care professionals, while specialized care includes secondary and tertiary care (see definitions below).

In Europe the term “nursing home” can refer to different models between and even within countries. For the purposes of this report, the term “nursing home” is understood to mean residential facilities that provide 24-hour nursing and social care to older people who require constant care and supervision.

The main reason across European countries for admission to a nursing home is dementia, followed by characteristics such as living alone, dependency in daily activities, and number of prescriptions (Voigt-Radloff et al., 2009; Spang et al., 2023). Other specific health problems in nursing home residents include visual and hearing impairments, malnutrition, oral health problems and mental health disorders, especially depression (Spreckelsen et al., 2020b; Nagel et al., 2024). Therefore, most people who are admitted to a nursing home are living with frailty, multimorbidity, polypharmacy and cognitive impairment (Czwikla et al., 2022). As women tend to get older than men, more women than men live in nursing homes. Most people admitted to a nursing home also die in the facility (Bollig, Gjengedal & Rosland, 2016; Fassmer et al., 2024).

LTC: a definition

LTC encompasses a broad range of services that provide social, personal and medical support to individuals with limited and/or declining functional ability due to mental or physical illness and/or disability (Eurostat, 2017; WHO, 2022a). These services aim to help individuals maintain a level of functional ability that is consistent with their basic rights and human dignity, while also alleviating suffering and maintaining their highest possible quality of life, independence, participation, equality and autonomy (Eurostat, 2017; WHO, 2022a). LTC is usually provided over extended periods of time, but the frequency and intensity of support required varies from person to person, depending on their needs (Eurostat, 2017; WHO, 2022a).

LTC services

The need for LTC is typically evaluated by an independent agency that determines which care services are necessary. LTC should be provided to anyone who requires assistance with activities of daily living (ADL) and/or experiences a decline in their intrinsic capacity (WHO, 2022a). This can happen at any point in life, but the need for LTC increases significantly with age (WHO, 2022a).

Historically, LTC has been provided by families, informal caregivers (particularly women) and communities. Nowadays, especially in high-income countries, LTC is becoming increasingly formalized in LTC settings and at home, provided by professional care workers (Eurostat, 2017; WHO, 2022a).

Formal LTC includes services to prevent, mitigate and rehabilitate functional decline (WHO, 2022a). As described above, it can be delivered in different settings, e.g., through support at home or in residential care facilities (WHO, 2022a). For example, inpatient LTC, which is mainly provided in nursing homes, covers nursing and/or personal care (Eurostat, 2017). LTC services are usually provided together with other services such as accommodation and support (Eurostat, 2017). The greater the dependency level, the more comprehensive are the care services offered (Eurostat, 2017). The various components of LTC comprise (Eurostat, 2017; WHO, 2022a):

1. medical and nursing care, i.e., preventive services, chronic disease management, rehabilitative care and maintenance of health status;
2. personal care services, which are usually provided by nursing staff as the need for personal care is often linked to underlying medical conditions. These include support with ADL such as eating, washing, dressing and managing incontinence;
3. assistance services, i.e., support with instrumental ADL such as doing laundry, housework, cooking and managing finances; and
4. other social care and well-being services, i.e., community activities and occupational support.

Financing of LTC

There are various forms of LTC financing and in most countries public coverage systems and private sources of funds co-exist and complement each other to varying degrees (Cylus et al., 2025).

Private LTC financing includes unpaid care, typically provided by informal caregivers, as well as the direct purchase of services, funded by the income and wealth of the care recipients (Publications Office of the European Union, 2021; Peña-Longobardo & Oliva-Moreno, 2022; Cylus et al., 2025). It also encompasses co-payments for partially publicly funded services, private LTC insurance and charitable spending on LTC (Cylus et al., 2025).

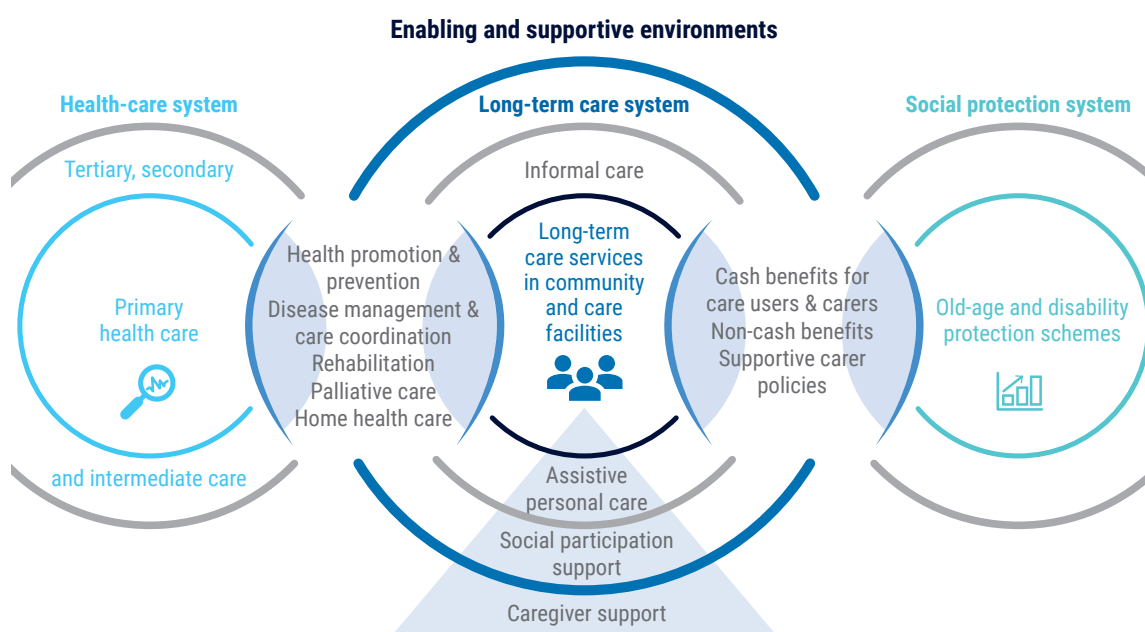
Public LTC financing refers to the public sectors' involvement in financing LTC, for example through social schemes, pensions or through the health system (Cylus et al., 2025). Universal LTC coverage usually provides publicly funded care for all eligible individuals according to their care needs, regardless of their financial status, and is typically funded through public social insurance, a mandatory tax on employment (with contributions from both employees and employers), and/or general taxation (Cylus et al., 2025). In contrast, means-tested LTC coverage only makes publicly

funded LTC available to those with the greatest needs and the fewest financial resources, as determined by income and/or assets. Those whose income and/or assets exceed a certain threshold (the means test) must cover the full costs of their care and often receive no support in finding and arranging suitable care (Cylus et al., 2025). Taxes levied by governments are the primary means of financing public LTC worldwide. Furthermore, social insurance is commonly used to fund pensions, though some countries also use it to fund healthcare and LTC (Cylus et al., 2025). However, even with a universal public LTC system, individuals in many countries are expected to make significant contributions to the costs of their care (Cylus et al., 2025). Therefore, private LTC insurance can complement the coverage provided by the public system and offer extra protection against risk (Cylus et al., 2025).

LTC systems

Formal care services, informal care and enabling environments, which ensure that people who experience a decline in intrinsic capacity can function independently for longer, should be the three key components of an LTC system (WHO, 2022a). As shown in Fig. 1, the functions of, and boundaries between, health, LTC and social protection systems, as well as between informal and formalized care and service provision, often overlap (WHO, 2022a). Furthermore, the importance of the three components of an LTC system vary depending on the country; worldwide, LTC is still mainly provided informally and without pay, especially in low- and middle-income countries (Eurostat, 2017; WHO, 2022a).

Fig. 1 The overlap between the health system, the LTC system and the social protection system



Source: WHO, 2022a.

Primary and specialized care: a definition

As illustrated in Fig. 1, primary care plays a central role in the provision of LTC services, as it involves services such as prevention (including vaccination and screening), health promotion, rehabilitation, disease management and palliative care, empowering individuals and communities to live healthier by being able to access prevention and early detection services, as well as treatment choices (WHO Regional Office for Europe, 2022; WHO, 2022; McKenzie et al., 2023). Primary care comprises general medical and public health services, usually provided by first-line health services, such as GPs and nurses (WHO, 2022; Papanicolas et al., 2022). These professionals are typically the first point

of contact for individuals seeking healthcare, managing common and unspecified illnesses, and referring patients to specialist care when necessary (WHO, 2022; Papanicolas et al., 2022).

Allied health professionals, such as physiotherapists and occupational therapists, contribute to primary and secondary care by providing preventive care and managing chronic diseases. For instance, physiotherapists help with mobility, movement, pain management and fall prevention; occupational therapists assist with everyday tasks; speech and language therapists support with speech, communication and swallowing difficulties; and dietitians provide nutrition interventions and nutrition care to prevent malnutrition (Bartels, Moak & Dums, 2002; Miller et al., 2020; Davis et al., 2022, 2024; Bartrim et al., 2023; Portillo, Calvo Arenillas & Miralles, 2023).

Specialized care refers to a) secondary care, which is usually provided in outpatient clinics or local hospitals and focuses on more complex medical conditions; and b) tertiary care, which is highly specialized care usually delivered in regional or national hospitals to treat more complex and costly conditions (Papanicolas et al., 2022). Specialized care for nursing home residents includes services provided by psychiatrists who manage psychiatric disorders and deliver mental health services (Seitz, Purandare & Conn, 2010).

However, it is important to note that the definitions and understandings of primary and specialized care vary between countries, and the boundaries between the two and between them and public health services are becoming increasingly blurred (Papanicolas et al., 2022).

Research aims and objectives

In light of the growing older population, the challenge of LTC has been recognized in many European countries (Rostad et al., 2020; Pedersen, Skinner & Sogstad, 2024). This report explores how selected countries in Europe (i.e., England, Germany, the Netherlands, Norway, Poland, Spain and Sweden) organize and fund the provision of primary and specialist health services to residents living in nursing homes that provide 24-hour nursing and social care.

Specifically, it provides information for each of the countries on:

1. the contextual background (in particular with regard to the organization and funding of residential LTC);
2. the provision of primary care to nursing home residents (with a focus on primary care physicians rather than nurses or care assistants);
3. access to allied health professionals (such as physiotherapists and occupational therapists);
4. access to outpatient specialized care (such as psychiatrists); and
5. needs assessment and referral.

A rapid literature search of the databases PubMed/MEDLINE Ovid and Embase, as well as of Google Scholar and Google, was performed. Relevant studies were identified through a search of keywords and MeSH (Medical Subject Headings) terms, including the following search terms: primary care, specialized care, specialist services, long-term care, nursing care, residential facilities, nursing homes and geriatric assessment. The Boolean operators “OR” and “AND” were used to combine the keywords and MeSH terms. The search equation used the title, abstract and keyword fields. The search was performed in English and preference given to studies published since 2015. In addition, reference lists of selected papers were hand-searched. The rapid literature search was complemented by inputs from national experts and practitioners.

The country selection was purposeful and followed an initial literature search considering available studies and data. Based on available literature, the following European countries were selected, as they represent a variety of health and LTC systems and different models of care provision in nursing homes: England, Germany, the Netherlands, Norway, Poland, Spain and Sweden.

3.1 England (United Kingdom)

Contextual background

Healthcare in England is largely publicly funded by the English National Health Service (NHS), which is free of charge at the point of use for anyone who is ordinarily resident, with some exceptions (Ball, Rafferty & Philippou, 2019; Santo, Conti & Wolters, 2020). However, (in contrast to Scotland) social care services in England are means-tested and only publicly funded if individuals and their families have an income and/or assets below £23 250 based on a financial assessment (Santo, Conti & Wolters, 2020; Hu et al., 2022; Cylus et al., 2025). If they are above the means-tested threshold, they have to pay the full cost of care by themselves, leading to around 41% of people living in care homes who self-fund their care (Hu et al., 2022; Cylus et al., 2025; UK Government, 2025).

If funded publicly, LTC is mostly financed by local authorities through local taxes, with local authorities setting their own charging policies, which can lead to inequalities across the country, and by central government grants (New York Times, 2023). Individuals can also apply for NHS-funded nursing care or NHS continuing healthcare, where the NHS contributes financially to nursing care costs in England for people with severe and primarily health-related care needs (Conroy et al., 2009; UK Parliament, 2023; Carers Trust, 2025). Access to LTC is determined by a needs and means assessment (Cylus et al., 2025).

There are two principal types of residential care home for older people in England: with or without nursing, with different regulatory staffing requirements. Residential care homes without nursing (also known as “residential homes”) offer personal care for ADL (Carehome.co.uk, 2025a). Residential care homes with nursing (also known as “nursing homes”) offer personal care and 24-hour assistance from qualified nurses (Carehome.co.uk, 2025a). There are also residential care homes with dementia care which often have nurses with dementia training. Finally, there are dual-registered care homes that provide both personal care and nursing care, depending on the needs of residents, which may change over time (Carehome.co.uk, 2025a).

Most residential LTC for older people with complex healthcare needs is provided by private for-profit nursing homes in England, which provide care for different requirements, such as dementia, while others cater for residents with a range of care needs (Honinx et al., 2021b; Carehome.co.uk, 2025b). So-called memory care units exist in some nursing homes, although it should be noted that specialized nursing homes are the minority in England (Senior Home Plus, 2025).

Access to primary care

In England GPs are responsible for coordinating and delivering medical care, and also act as gatekeepers, referring nursing home residents to specialist and hospital services (Chadborn et al., 2021a; Sattar et al., 2023). GPs are among the health professionals who visit nursing homes most frequently and provide the first point of contact for most residents’ health needs (Sattar et al., 2023).

The way in which GPs work with nursing homes varies depending on the region and local practices (Chadborn et al., 2021b). This includes single GP practices working with single nursing homes, GPs working across several nursing homes, and even (in rare cases) nursing homes owned by GP practices. People moving into a nursing home may keep their original GP if the nursing home is in the catchment area of the GP practice, but many nursing homes have established links with specific GP practices and ask their residents to register with these practices, which visit the nursing homes regularly. This means, however, that in some cases different GPs are attending different residents in a single nursing home.

GPs are reimbursed for their work with nursing home residents in the same way as they are paid for providing care to people living in their own homes, according to the NHS General Medical Services contract. To emphasize regular contact with nursing homes, some institutions offer additional financial incentives to GPs, or GPs partner with selected nursing homes (Gordon et al., 2018; White & Alton, 2022), based on Personal Medical Services and Local Enhanced Service contracts.

Most medical care is provided by GPs (Conroy et al., 2009) and GP visits are requested by the nursing home and/or GPs pay regular scheduled visits (White & Alton, 2022). Community pharmacies provide medication support services to nursing homes, including medication management, auditing, and training on medication administration.

Access to allied health professionals

Most direct care in English nursing homes (approximately 80%) is delivered by care workers and senior care workers, who are supported by visiting primary care physicians and professionals, community-based nurses and therapists (Gordon et al., 2018; Griffin et al., 2024). Most health professionals are employed on either a part-time or full-time permanent basis within the NHS, and work at least for part of their role in a community setting (Davis et al., 2024).

In general, health services are provided within the nursing home, with allied health professionals who are employed by the NHS or are self-employed visiting the nursing home residents. Physiotherapy and rehabilitation, occupational therapy, and speech and language therapy are additional services in nursing homes in England, which vary by facility (Senior Home Plus, 2025). For example, speech and language therapists may be employed by nursing homes on a permanent basis or collaborate with a nursing home, visiting between one and five nursing home residents in an average working week, mainly due to referrals regarding swallowing or communication needs (Davis et al., 2024). If the nursing home does not offer these services, residents can access NHS community services upon referral by their GP, or pay for private services that come into the nursing home.

Access to specialized care

Most nursing homes in England employ care workers who may be trained in areas such as dementia and palliative care (Gordon et al., 2018). Based on data from 2023/24, most of the adult social care workforce has received training in “moving and handling”, “safeguarding adults” and “infection control” (Skillsforcare, 2025). However, more than half of the adult social care workforce (excluding regulated professionals) lacks a relevant social care qualification (Skillsforcare, 2025).

Other services provided by nursing homes in England comprise enhanced primary care support (hydration and nutrition support and oral care), wound care, rehabilitation, prevention of falls, palliative care, mental health care and dementia care, based on the recently introduced *Enhanced Health in Care Homes* model, designed to improve healthcare for nursing home residents by integrating primary, community and specialist services (NHS England, 2023; Attwood et al., 2024).

While palliative care is mostly provided within the NHS and the charitable sector, specialist palliative care advice is also provided for professionals delivering palliative care in the nursing home (Honinx et al., 2021b). Furthermore, multidisciplinary meetings are held in most nursing homes, with the frequency differing depending on the facility (Honinx et al., 2021b).

Needs assessment and referral

Personalized care and support plans are based on the principles and domains of comprehensive geriatric assessment (CGA) and structured medication reviews, which take place upon admission of a new resident to the nursing home (NHS England, 2025). Furthermore, allied health professionals, such as speech and language therapists, may also assess nursing home residents, for example looking at cognition, repetitive/functional language, emotional well-being, symptoms of dementia and social communication (Davis et al., 2024).

Nursing homes in England rely on primary care to provide medical care and access to specialist healthcare services. As described above, care home staff initiate and negotiate access to primary and

specialist care on behalf of their residents, while GPs act as the leaders and coordinators of residents' medical care (Chadborn et al., 2021b, 2023; Warmoth, Aylward & Goodman, 2024). Alternatively, nursing home residents can self-refer to a social service by contacting their local council to arrange an appointment, or arrange a private consultation with an independent health professional via a website. This consultation will incur a charge (RCOT, 2025).

3.2 Germany

Contextual background

Germany's health system is mainly based on a mandatory social health insurance system, characterized by competing non-profit sickness funds (Schwettmann et al., 2023). There is a mandatory LTC insurance system, financed through income-dependent membership contributions (WHO, 2022a; Skudlik et al., 2023; Fassmer et al., 2024). However, LTC insurance covers only part of the actual costs incurred, while the rest is paid either by private co-payments or, in case of financial need, by social welfare (WHO, 2022a; Schwettmann et al., 2023; Fassmer et al., 2024). Most of the costs of nursing and medical care in nursing homes are covered by the LTC insurance system (Verbraucherzentrale, n.d.; Ågotnes et al., 2019; Gesund.bund.de, 2025a).

To receive benefits from the LTC insurance in Germany, the need for care must be assessed by an assessor from the Medical Service (*Medizinischer Dienst*), which assigns a care grade (1–5) to the person applying (Fassmer et al., 2024). From care level 2 onward, the LTC insurance pays a monthly subsidy, with the amount increasing with the time spent in a nursing home (Verbraucherzentrale, n.d.; Gesund.bund.de, 2025b). For those living in nursing homes, the subsidy covers part of the costs of the facility. Residents also pay a standardized co-payment, which is the same for all residents within one care facility (ZQP, 2023a; BMG, 2025; Gesund.bund.de, 2025a).

Most nursing homes in Germany are run by non-profit organizations (53%), followed by for-profit (42%) and public providers (5%) (Ågotnes et al., 2019). The medical and nursing care offered and the services provided by nursing homes depend on each German federal state and are specified in state framework agreements between care providers and care insurance companies (ZQP, 2023a). Many care homes in Germany specialize in certain areas of care, such as severe dementia or special medical needs, however, in most care homes, a wide range of morbidities exists (ZQP, 2023a; Fassmer et al., 2024). Around a third of facilities has at least one psychogeriatric living area (for the care of dementia and other psychiatric disorders) (Fassmer et al., 2024). Usually, short-term and long-term care are available (Ågotnes et al., 2019).

Access to primary care

Nursing homes provide 24-hour full-time residential care by trained nursing and care staff but medical care is primarily delivered by external GPs who are mainly self-employed, providing nursing home visits (Müller et al., 2018; Spreckelsen et al., 2020a; Gesund.bund.de, 2025a). Residents can choose their GP freely and most primary care physicians continue to care for a patient after admission to a nursing home (Ågotnes et al., 2019; Schwettmann et al., 2023). On average, GPs in Germany care for 47 residents in four different nursing homes, in addition to working in their own practice, while German nursing homes are on average served by nine different GPs with a large variance among the different facilities (Schröder et al., 2020; Schwettmann et al., 2023). However, the general availability of GPs is often limited in nursing homes (Ågotnes et al., 2019). Outside doctors' consulting hours, patients can use the acute out-of-hours medical care, provided by the associations of statutory health insurance physicians in each of the German federal states (Fassmer & Hoffmann, 2020).

Medical care services in Germany are paid for and provided by the National Association of Statutory Health Insurance Funds (*GKV-Spitzenverband*) and the National Association of Statutory Health Insurance Registered Doctors (*Kassenärztliche Bundesvereinigung*) (Ågotnes et al., 2019). Nursing

homes are obliged to have cooperation agreements in place with GPs, which are funded by the health insurance funds (Gesund.bund.de, 2025a). Physicians are reimbursed through a fee-for-service model, though billing is capped and only certain types of medical services qualify for reimbursement (Ågotnes et al., 2019).

Nurses and nurse assistants work in nursing homes mostly part-time, while nurse practitioners do not yet exist in the German health system (Müller et al., 2018). Both general nurses and geriatric nurses are employed in nursing homes (Reese et al., 2021). Nurses are only responsible for the basic nursing care of residents, such as assistance with mobilization or personal hygiene; a physician must explicitly delegate all procedures concerning medical care and medication (Müller et al., 2018; Reese et al., 2021).

Access to allied health professionals

According to research studies, physiotherapists and speech therapists are present in almost all German nursing homes and the need for these professions is high (Schröder et al., 2020; Fassmer et al., 2024). However, only about a third of German nursing homes has permanently employed allied health professionals, with occupational therapists (34%) and social workers (29%) being the most common groups representing permanent staff, while most allied health professionals work in private practices and offer home visits to nursing homes based on a physician's prescription (Schwettmann et al., 2023; Fassmer et al., 2024). Care delivered by allied health professionals is publicly covered if it is medically necessary and prescribed by a doctor, aimed at maintaining or improving the abilities of nursing home residents, or arranged by the nursing home directly (KVBB, 2024; Gesund.bund.de, 2025c). In these cases, the statutory health insurance, the LTC insurance or the nursing home pay for these services, although residents have to make co-payments (10% of the cost of therapeutic services and €10 per prescription) (KVBB, 2024; Gesund.bund.de, 2025c).

Access to specialized care

The availability of specialists is often limited in German nursing homes, but care facilities are legally required to have cooperation agreements with specialists, dentists and pharmacies in place (Ågotnes et al., 2019; Gesund.bund.de, 2025a). Furthermore, nursing homes are expected to also collaborate with other medical and therapeutic offices, as well as with hospitals and hospice services (ZQP, 2023a, 2023b).

Specialists usually only work in private practices or hospitals and may offer home visits to nursing homes (Schwettmann et al., 2023; Fassmer et al., 2024). While almost all German nursing homes are in contact with all medical specialties, the availability of medical specialists is much lower than that of GPs (Schröder et al., 2020; Schulz et al., 2020a, 2020b; Czwikla et al., 2022). A possible lack of mobile diagnostic equipment might be linked to a gap in care consisting of regular visits of specialists, such as dentists and ophthalmologists (Schröder et al., 2020; Czwikla et al., 2022). Therefore, transport services and medical escort services to visit specialists off-site are available in some nursing homes, which are usually covered by the health insurance (Spreckelsen et al., 2020b; Czwikla et al., 2022). In general, German nursing homes report a high demand for care from psychiatrists, neurologists, urologists and dentists (Schröder et al., 2020; Spreckelsen et al., 2020b; Fassmer et al., 2024). However, specialist contact often depends on whether a resident had such care prior to entering the nursing home, with some increases in visits after entering the nursing home (for example, neurologists, psychiatrists) and decreases in visits in others (for example, ophthalmologists, surgeons) (Spreckelsen et al., 2020a).

Palliative care is mainly provided by nursing staff and GPs, with specialized ambulatory palliative care available upon prescription (Rehner et al., 2025).

Needs assessment and referral

Access to GPs and medical specialists for nursing home residents is primarily organized by nursing home staff (Czwikla et al., 2022). Specialist contacts mainly occur after referral by GPs and is highest for neurologists and psychiatrists in German nursing homes (Schröder et al., 2020; Spreckelsen et al., 2020a). Physiotherapy, occupational therapy and speech therapy treatments must also be prescribed by a physician (Fassmer et al., 2024). However, nursing homes or residents themselves can also independently consult specialists (Fassmer et al., 2024). As no gatekeeping system of GPs is established on a mandatory basis, nursing home residents can directly access medical specialists in the ambulatory sector (Spreckelsen et al., 2020a).

3.3 The Netherlands

Contextual background

The Netherlands was one of the first countries to develop and implement LTC programmes for older people and is considered a pioneer in the provision of care for older people (Hospers, Chahine & Chemali, 2007; van de Haar & Alme, 2025). It has a mandatory social health insurance system as well as a mandatory LTC insurance system, with contributions based on income (Grol et al., 2021; Schwettmann et al., 2023; van de Haar & Alme, 2025). The LTC insurance system covers the support and care services for those who require permanent supervision or 24-hour care, such as nursing home expenses for the most vulnerable older people, including 24-hour multidisciplinary, personalized treatment for severe and complex health problems (Koopmans, Pellegrom & van der Geer, 2017; Meershoek, Broek & Crea-Arsenio, 2022).

The need for LTC is assessed by an independent central agency, which assigns individuals to one of ten care packages included in the LTC insurance, of which eight are for LTC and two are for short-term care provided in LTC facilities (Government of the Netherlands, 2017; Ministerie van Volksgezondheid, Welzijn en Sport, 2020; Meershoek, Broek & Crea-Arsenio, 2022; Schwettmann et al., 2023). Nursing home residents have to contribute towards the costs of their care (based on their personal circumstances, for example, age, income, assets, etc.) and pay for products and services that are not specifically necessary for 24-hour care (Conroy et al., 2009; Government of the Netherlands, 2016a, 2016b). In private nursing homes, which do not receive money from the government, all costs of care have to be paid for by the residents themselves (Government of the Netherlands, 2016a).

All nursing homes in the Netherlands are private entities. Those that are fully publicly funded, where total costs are reimbursed by the LTC insurance, are not allowed to be for-profit, that is, they are not allowed to distribute profits to owners or stockholders. Conversely, partly privately funded nursing homes, where the costs of room and board are privately funded, can be for-profit (Bakx, Schut & Wouterse, 2020). Approximately 92% of Dutch nursing homes are run by non-profit operators (Fassmer et al., 2024). Dutch nursing homes are divided into different wards, which offer specialized care, including chronic psychogeriatric care (dementia and cognitive disorders), chronic somatic care (physical disabilities), geriatric rehabilitation and palliative care (Van Buul et al., 2020; Honinx et al., 2021a; Schwettmann et al., 2023; Fassmer et al., 2024).

Access to primary care

In contrast to other European countries, Dutch nursing homes traditionally employ specially trained ECPs, who have completed a three-year specialist training, with one full-time nursing home physician caring for approximately 100 residents (Meershoek, Broek & Crea-Arsenio, 2022; Vrijmoeth et al., 2022; Schwettmann et al., 2023). ECPs are employed directly by the nursing home and provide proactive on-site care (Koopmans, Pellegrom & van der Geer, 2017; Schröder et al., 2020; Ter Brugge et al., 2022; Fassmer et al., 2024). However, especially in smaller nursing homes or former residential homes, which were transformed into nursing homes after the implementation of the LTC insurance system, primary care may also be provided by GPs.

Access to allied health professionals

In addition to ECPs, GPs and nursing staff, almost all Dutch nursing homes have permanently employed allied health professionals, providing care services on site, with psychologists, physiotherapists, occupational therapists, speech therapists and nutritionists being the most common ones, together forming a multidisciplinary care team and allowing for adequate, continuous care (Schols, Crebolder & van Weel, 2004; Sinoo et al., 2015; Van Buul et al., 2020; Meershoek, Broek & Crea-Arsenio, 2022; Schwettmann et al., 2023). Physiotherapists, for example, work around 20 hours per week in nursing homes, providing 55 minutes of care per week to each of their patients, based on a survey (Leemrijse et al., 2007; Sterke et al., 2021). Most medical care is provided in primary care services by the ECP and allied health professionals during office hours and on-call, as well as by nursing staff on site (available 24-hours), while specialist medical care is available on referral from the ECP in a polyclinic, hospital or also on site, if possible (Conroy et al., 2009; Meershoek, Broek & Crea-Arsenio, 2022; Fassmer et al., 2024).

Access to specialized care

The extensive training of ECPs allows them to meet their patients' basic needs in neurology, psychiatry, ophthalmology and otolaryngology (Fassmer et al., 2024). In addition, geriatricians work closely together with nursing homes' ECPs or GPs, providing elective admissions and outpatient services for more detailed diagnostic evaluation, as well as visiting patients in the nursing home, when needed (Conroy et al., 2009). Furthermore, because Dutch nursing homes combine the services of intermediate care, community hospitals and services, day care and hospice in one setting, many medical services such as intravenous fluids, nasogastric feeding and antibiotic therapy can be provided within the nursing home, and many acute problems can be treated on site, thus avoiding acute hospital admissions (Conroy et al., 2009).

Box 1 The work of ECPs in the Netherlands

In the Netherlands medical care for elderly and vulnerable people in nursing homes is traditionally the responsibility of ECPs. This physician specializes in – and specifically focuses on – vulnerable elderly individuals and those with chronic conditions or disabilities, whose independence is reduced due to one or more conditions, limitations or disabilities. This is characterized by interrelated conditions (multimorbidity) and the use of multiple medications (polypharmacy).

The ECP has the expertise to provide complex medical care, taking into account life expectancy and personal wishes regarding life and end-of-life care of patients. They are responsible for both GP care and specific nursing home care. The approach is holistic, with a multidisciplinary team of healthcare professionals, psychologists and paramedics in the nursing home to whom the ECP can refer using the electronic health record. The goal is to enable patients to function optimally, taking into consideration personal needs and wishes, somatically, psychologically, socially and spiritually.

ECPs see their patients face to face frequently. If necessary, they involve legal representatives and/or consult medical specialists in the hospital or fellow ECPs. They develop, implement, monitor, evaluate and adjust treatment on the basis of shared decision-making with patients. In their organizational context, they are involved in medical management tasks and policy development. In addition, ECPs can participate in regional and national networks for nursing home care and, as certified educators, they can be involved in training young doctors and future ECPs.

Patients of ECPs may be in multiple locations, each with its own dynamics and core characteristics: a large nursing home, a small-scale residential facility or elderly frail people

continued on next page

Box 1 *continued*

in need of care living at home. Care types include somatic and psychogeriatric care, geriatric rehabilitation, recovery-oriented care, temporary care and palliative care. Each care type requires specific knowledge and expertise and has its own specific funding requirements. In residential care facilities, or when individuals are living at home, the GP provides general care, while the ECP may be consulted by the GP for specific problems or may be appointed as co-practitioner. For specific care, such as geriatric rehabilitation, palliative and psychogeriatric care, ECPs may follow additional training, providing substantive added value to both the patient and the nursing home. ECPs may also conduct scientific research.

Overall, the added value of the ECP as a coordinating practitioner lies primarily in their broad perspective. While a GP works mainly on a demand-driven basis, the ECP works more proactively by connecting the dots, continually weighing the pros and cons of treatment, and making choices for and with patients, with the overall aim of achieving personalized care.

Source: Wietske Berrevoets

Needs assessment and referral

In the Netherlands nursing homes make extensive use of geriatric assessment, especially of care needs, and aim for continuity of care, using a person-centred approach (Conroy et al., 2009). The ECP or GP is responsible for developing and evaluating an integrated care plan for each nursing home resident, which is formulated after admission and after a comprehensive and thorough assessment of the resident's health problems (Schols, Crebolder & van Weel, 2004; Sinoo et al., 2015). Based on the resident's care plan, the effectiveness of the care is regularly evaluated and revised, if necessary (Schols, Crebolder & van Weel, 2004).

Depending on the case, other specialists, such as physiotherapists, may also see newly admitted patients without a specific referral – or they may see the resident a few weeks after admission by referral of the nursing staff, allied health professionals, or unsolicited (Sterke et al., 2021). Assessment of the need for health services, such as physiotherapy or nutritional care, is done using standardized instruments, screening tools and clinical markers, such as weight, but also by observing both residents and nursing staff performing ADL without a specific instrument to observe triggers and interactions (van Nie-Visser et al., 2011; Sterke et al., 2021). The ECP or GP supervises the multidisciplinary care team in the provision of care and case conferences are held regularly for each patient (Conroy et al., 2009; Sinoo et al., 2015).

3.4 Norway

Contextual background

In Norway municipalities are responsible for providing primary care and LTC. Funding for LTC comes predominantly from the public sector, based on grants from the central government and taxes on income and wealth (Glette et al., 2018; Ågotnes et al., 2019; Theisen, 2020). User fees for nursing homes mainly depend on the resident's taxable income from pensions. While user fees for short-term nursing homes are limited to NOK 200 per day (approximately €17), residents of long-term nursing homes may pay up to 85% of their income, although not more than the actual cost of care (Glette et al., 2018; Theisen, 2020). The need for LTC is assessed by municipalities (Pedersen, Skinner & Sogstad, 2024).

Most nursing homes in Norway are publicly owned and operated, while a minority of nursing homes are private non-profit, and only a few are private for-profit (Lund et al., 2023). However, the municipalities are legally responsible for providing the “necessary medical care” to individuals residing within their borders, regardless of the nursing home's ownership model (Ågotnes et al., 2019).

Most Norwegian nursing homes offer long-term and short-term units, which are divided into different specialized wards (Glette et al., 2018; Ågotnes et al., 2019). For example, some nursing homes have short-term beds available for rehabilitation and palliative care, while others offer long-term psychogeriatric wards for patients with advanced dementia and/or with special needs (Rostad et al., 2020; Van Buul et al., 2020). Especially specialized dementia care is highly prevalent in Norwegian long-term nursing home stays, representing the majority of nursing homes (Ågotnes et al., 2019; Rostad et al., 2020; Lund et al., 2023).

Access to primary care

Although there are no formal educational requirements for working in Norwegian nursing homes, all employees must have the necessary qualifications to work in healthcare. All employees must receive training, which must be documented within the institution.

In Norway medical care in nursing homes is provided by physicians, mostly GPs, who are employed by the municipalities and combine this part-time work with work in their practice (Glette et al., 2018; Ågotnes et al., 2019; Van Buul et al., 2020; Christensen et al., 2022). In this case, GPs tend to visit the nursing home at least one day a week (Van Buul et al., 2020; Christensen et al., 2022). While the municipalities are responsible for offering GP services within their facilities, nursing home residents can also choose to keep their own GP. Physicians are responsible for all patients in the nursing home. If a patient has chosen to keep their GP, nursing home physicians must offer dialogue and services in cooperation with that practitioner. Some nursing homes also employ their own dedicated ECP who works at the facility full-time, depending on the type of nursing home and its location.

In Norway there are no national regulations stipulating the minimum coverage required of nursing home physicians (for example, the minimum frequency with which they must contact residents), how they are employed (for example, whether they work directly for the nursing home or for the municipalities as a GP), or how they are reimbursed (Ågotnes et al., 2019). Physicians employed by individual nursing homes or municipalities receive a fixed salary, regardless of the number and type of patients they treat, and nursing homes are reimbursed by the municipality for their physicians' salary costs (Ågotnes et al., 2019). Salary levels for physicians in the public sector are also highly regulated, so differences in salary between nursing home- and municipal-employed physicians are minimal in most cases (Ågotnes et al., 2019).

Nurses are available 24-hours and usually responsible for multiple wards during evenings, nights, weekends and holidays (Van Buul et al., 2020). Therefore, they are not always present in one nursing home as they work within and between municipalities (Glette et al., 2018; Van Buul et al., 2020). If the nursing home physician or GP is not available, the municipal emergency room doctor is often responsible for the residents, for example, during nights and weekends (Glette et al., 2018).

Access to allied health professionals

In general, there is no mandatory staff-to-resident ratio or standards for staff qualifications in Norway, nor are there any requirements regarding skill-mix within nursing home care teams (Lund et al., 2023). However, all nursing homes must employ an administrative manager, a physician and a nurse responsible for medical care. Furthermore, they must also have a sufficient number of healthcare personnel to provide necessary care and assistance, depending on the size of the nursing home.

The nursing home staff who provide direct patient care include registered nurses, licensed practical nurses, nurse assistants with no formal education, and social educators/social workers (Glette et al., 2018; Lund et al., 2023). Furthermore, most nursing homes have physiotherapists in their treatment team, especially in the rehabilitation wards (Van Buul et al., 2020). If a resident requires occupational therapy, this is usually provided in the nursing home by publicly funded municipal health services.

Access to specialized care

Norway offers different specialized LTC services for people living in nursing homes (Sogstad, Hellesø & Skinner, 2020). However, the level of specialization and the range of care service models used vary from a more generalist approach with a limited number of specialized services to highly specialized models, focusing on dementia care, palliative care and rehabilitation (Sogstad, Hellesø & Skinner, 2020).

Needs assessment and referral

Upon admission, an admission interview takes place with a physician to assess the resident's care needs and potentially also talk about advanced care planning, while mainly focusing on practical issues and the patient's life story (Gjerberg et al., 2017; Thoresen et al., 2019). If the nursing home resident's health deteriorates, further conversations take place (Gjerberg et al., 2017).

3.5 Poland

Contextual background

Almost the entire population is insured by the mandatory social health insurance system (Windak & Oleszczyk, 2015). LTC in Poland is provided by the healthcare, social and private sectors, and includes cash and in-kind benefits (Wrotek & Kalbarczyk, 2023; Ujazdowski, 2025). Depending on whether LTC is provided within the health system or the social care system, the provision and financing of care may differ. Providers within the health system include residential care facilities such as "chronic medical care homes" (*zakład opiekuńczo-leczniczy*), nursing homes (*zakład pielęgnacyjno-opiekuńczy*) and psychiatric nursing homes (*zakład pielęgnacyjno-opiekuńczy psychiatryczny*). By contrast, residential homes (*dom pomocy społecznej*) belong to the social welfare system and have less of a medical focus than other forms of residential care facilities.

Medical treatment and health services in nursing homes are covered by the health insurance system (Golinowska, 2010). The fees paid by the nursing home residents cover the costs of accommodation and meals, capped at 70% of their income (Wrotek & Kalbarczyk, 2023). Different cash benefits are available for people aged 75 years or older, as well as for younger people who are fully dependent on care and have a disability. Formal LTC is currently very poorly staffed in Poland due to a lack of care workers and low levels of remuneration (Wrotek & Kalbarczyk, 2023; Ujazdowski, 2024). Furthermore, there is still a high supply of informal care, such as care through family members, and a low supply of formal care, such as institutional care options (Wrotek & Kalbarczyk, 2023).

The most common type of nursing home in Poland is publicly owned and non-profit, followed by privately owned and non-profit models (Honinx et al., 2021b). There are also private for-profit nursing homes (Honinx et al., 2021b).

Access to primary care

All nursing homes must have 24-hour nursing care available and provide physician care on site a minimum of twice a week and on call (Baranowska-Tateno et al., 2024). Usually, nurses and care assistants provide care on site, while physicians provide off-site care, as nursing home residents are normally registered with a particular GP of their choice in a local primary healthcare centre, where they visit their physician (Windak & Oleszczyk, 2015; Honinx et al., 2021b). However, models also exist where physicians provide on-site care and visit their patient in the nursing home on a regular basis or on demand (Van Buul et al., 2020).

Primary care physicians can work as either salaried employees or as independent contractors (Windak & Oleszczyk, 2015). The income of self-employed physicians mainly comes from capitation fees adjusted for factors such as age, chronic cardiovascular disease, diabetes or living in a residential home (Windak & Oleszczyk, 2015). Other payment methods within primary care include payment per consultation or visit, for example for services within cardiovascular disease prevention programmes.

Since 2022 the fee-for-service model has been used more extensively in Poland to pay for services within a coordinated care model for the most common chronic conditions, as well as for new diagnostic tests that expand the scope of primary care.

The majority of staff working in Polish nursing homes is nursing personnel, including LTC nurses who hold a special qualification, followed by nursing assistants, medical workers, who undergo vocational education or a one-year post-secondary school degree, and physicians (Golinowska, Kocot & Sowa, 2014; Świtalski et al., 2023). Because of a shortage in qualified staff, including geriatricians, nurses often have to make decisions independently (Ujazdowski, 2024).

Access to allied health professionals

In addition to physicians and nurses, other professionals providing care in Polish nursing homes include educators, social workers, medical workers, physiotherapists, psychologists, speech therapists, occupational therapists and dietitians (Golinowska, 2010; Van Buul et al., 2020; Kijowska et al., 2021; Świtalski et al., 2023). Physiotherapy, for example, is provided for free to nursing home residents, if they have a referral from their GP and if their nursing home has a contract with the National Health Fund. Some nursing homes have in-house physiotherapists, while others work with external providers. While multidisciplinary meetings are held in the majority of nursing homes, their frequency differs, depending on the facility (Honin et al., 2021b).

Box 2 Report by a dietitian providing health services to nursing home residents in Poland

Our nursing home has a contract with the National Health Fund and we have capacity for 120 residents. We admit residents with a score of up to 40 points on the Barthel scale, which is used to measure performance in activities of daily living and is a requirement for funding from the social health insurance system.

We have our own kitchen in which we cook all meals for our residents. In Poland the institution confirming the competences to practise the profession of dietitian is the Ministry of Health. In our nursing home dietitians, chefs and kitchen assistants work 8- or 12-hour shifts from Monday to Friday (occasionally weekends or holidays).

Residents have different dietary needs, for example for diabetic, high-protein, dairy-free or mixed diets. Each day I have a briefing with a nurse and a doctor to determine if any patients require changes in their diet since the previous day. The diet list is passed on to the kitchen every day for the following day.

When we have a new patient admission, I conduct an interview (if possible), read the doctor's recommendations and assign the patient to a specific type of diet. In my workday I prepare a menu for two weeks, taking into consideration holidays and other special occasions. It is also my duty to prepare price calculations for these menus and food rations. Each diet must be calorie-balanced.

Challenges include that geriatric patients have no appetite or have unhealthy eating habits. Moreover, families may bring in food that is not healthy and not necessary, such as sweets, fatty foods, sweet drinks or food the patient may choke on. This is why from time to time I talk to patients and families about nutritional education. Sometimes I can participate in feeding a patient.

Apart from these tasks I work in the kitchen and help kitchen workers with food preparation. I also place orders for food products and ensure that all processes are carried out in accordance with standard procedures, for example hazard analysis and critical control points, a systematic approach to ensure food safety. My shift ends at 3 p.m. or 7 p.m., depending on the schedule.

Source: Agata Bakońska

Access to specialized care

In terms of specialized care, geriatric rehabilitation can be provided at medical nursing homes and is financed from the state or paid for privately by the patients (Sagan et al., 2022). However, geriatric care remains largely underdeveloped in Poland, especially in rural areas (Sagan et al., 2022; Supreme Audit Office, 2022). Furthermore, differences between nursing home types are often limited to specialist palliative care advice and certain specialized equipment being available (Honinx et al., 2021b). Enteral or parenteral nutrition is covered by social health insurance, if needed (Schubert, Czech & Skrekowska-Baran, 2018). To offer such services, providers must meet certain criteria, such as staff qualifications, medical equipment and time availability, which are specified in the benefit packages of LTC services in the health system.

Needs assessment and referral

Prior to admission to the nursing homes, people are tested using the Barthel Index and they must have a score of 40 points or less on the level of dependence scale (Świtalski et al., 2023; Wrotek & Kalbarczyk, 2023). Furthermore, CGA is considered an effective way of identifying care needs, which forms the basis for planning and establishes a care framework in Polish nursing homes, as well as taking into account the perspective of the nursing home resident and their caregivers (Mazurek et al., 2015). However, this approach is more theoretical and, in reality, used rather infrequently in Polish nursing homes.

In Poland every citizen is generally required to register with a GP, who acts as a “gatekeeper” to further medical services (Welcome Point, 2025). When referring a patient to a specialist, a primary care physician must issue a referral letter (Windak & Oleszczyk, 2015; Sowada, Sagan & Kowalska-Bobko, 2019). In some cases, no referral letter is required, for example in case of visiting a gynaecologist, dermatologist, ophthalmologist, psychiatrist or oncologist, even though these specialists are not recognized as primary care specialists (Windak & Oleszczyk, 2015; Sowada, Sagan & Kowalska-Bobko, 2019).

3.6 Spain

Contextual background

In Spain’s decentralized health system, the national government is responsible for the overall coordination of the health system, and the country’s 17 autonomous communities are in charge of the planning, financing and provision of social services, healthcare and education, offering different levels of protection and funding (Bernal-Delgado et al., 2024). Healthcare is mainly funded through general taxation (Bernal-Delgado et al., 2024).

Spain’s LTC system is part of the welfare state and highly decentralized, with variation between autonomous communities and even between municipalities within the same autonomous community (Vilaplana-Prieto et al., 2023). While recent policy changes aim to support formal care for older people, much of the burden still falls on families, especially women, and informal caregivers (Díaz-Tendero & Ruano, 2024; Campoy-Vila et al., 2025). Furthermore, access to care is often influenced by family income and the older person’s pension (Lillo-Crespo & Riquelme, 2018). This fact, coupled with the decentralization of the organization and management of care services for people in need, leads to inconsistent standards and funding, regional inequalities and a lack of consensus on care policies for older people (Lillo-Crespo & Riquelme, 2018; Díaz-Tendero & Ruano, 2024).

Access to LTC services is prioritized according to the applicant’s degree of dependency and financial assets. Services are co-paid according to the type of service required and the applicant’s ability to pay (Bernal-Delgado et al., 2024).

Nursing homes in Spain are either public, subsidized or private, with 23% of nursing homes in Spain being publicly owned (Campoy-Vila et al., 2025). Most LTC facilities in Spain are run by the

private sector and typically function as assisted-living homes (care homes), with some of these facilities providing more intensive nursing care (nursing homes). Each autonomous community has established its own requirements that care and nursing homes must meet (Vilaplana-Prieto et al., 2023). Organizational models can vary depending on the ownership of the nursing home, with private sector facilities tending to be assisted-living facilities and public ones tending to be nursing homes providing 24/7 care (Serrano-Urrea et al., 2017). In general, residential models in Spain primarily provide social services to older people, with health professionals responsible for providing most health services (Vilaplana-Prieto et al., 2023; Campoy-Vila et al., 2025). Overall, a need to improve the coordination between health services and care homes has been identified (Organización Médica C, 2020; Oliva & Peña Longobardo, 2022).

Access to primary care

In Spain primary care is generally provided by public providers, that is, the NHS. These providers consist of specialized family doctors and staff nurses who form so-called primary healthcare teams, which conduct planned and emergency visits, as well as patient-requested visits at the primary healthcare centre or on-site (Bernal-Delgado et al., 2024). However, although primary healthcare teams are required to conduct home visits, their usual workload often prevents them from doing so. The same is true for nursing home residents, who are generally registered with the local public health centre closest to the nursing home, although some may retain their previous GP.

Physicians are not usually employed directly by the nursing home (Vilaplana-Prieto et al., 2023). In the private sector, however, which owns most care facilities in Spain, the companies that manage these facilities may employ doctors directly or hire self-employed physicians. Generally, there is significant heterogeneity in the degree to which nursing homes are medicalized, that is, the medical services that are provided to nursing home residents within the facility, and data are lacking on the number of primary care doctor and nurse visits to nursing homes (Regato-Pajares et al., 2023). Except in the case of large nursing homes, it is common for a single doctor to provide health services to more than one nursing home, with the support of nursing assistants (Vilaplana-Prieto et al., 2023).

Requirements concerning the availability of physicians in nursing homes differ between the autonomous communities. For example, the Valencian Community requires physicians to spend a minimum of 5 hours per week in nursing homes with fewer than 100 places, and a minimum of 10 hours per week in facilities with more than 100 places, while the Basque Country has established a doctor-to-resident ratio based on each nursing home resident's level of dependency (Vilaplana-Prieto et al., 2023).

To strengthen professional LTC services, especially those provided in nursing homes, and to advance person-centred care, the Spanish Parliament is currently debating a reform of the *Dependency Law*. One focus of the planned reform is to improve the ratio of professionals to residents in Spanish nursing homes.

Nursing care is available 24-hours in Spanish nursing homes, provided by nurses and nurse assistants, while physicians and/or qualified geriatric nurses are available on call (Rodríguez-Martín et al., 2013; Blanco-Donoso et al., 2022; Campoy-Vila et al., 2025). Furthermore, nursing home primary care professionals (that is, physicians, nurses and assistant nurses) can call for emergency services at any time. If deemed adequate, they can transfer a resident to the emergency ward of the corresponding hospital.

Access to allied health professionals

Social workers, psychologists, occupational therapists and physiotherapists are available in almost all Spanish nursing homes (Blanco-Donoso et al., 2022; Riquelme-Marín et al., 2022). However, the availability of health professionals varies between nursing homes and regions. In most care homes, nurses are only responsible for tasks related to medication administration because of legally restricted nursing competencies, while physicians and allied health professionals are responsible for the remaining care duties (Lillo-Crespo & Riquelme, 2018). Depending on the ownership, nursing home staff may be employed directly by the public administration (in publicly owned and operated nursing homes), by the managing entity or through contracts with external providers (in subsidized nursing homes), or by a private company (in private nursing homes) (Fité-Serra et al., 2019; Campoy-Vila et al., 2025).

Box 3 Provision of physiotherapy services to nursing home residents in Spain – example: Community of Madrid

In Spain residents of nursing homes are first assessed by a doctor, then by a nurse, and then, where indicated, by a physiotherapist. Physiotherapy services are always provided in nursing homes, not outside them.

The initial physiotherapy assessment takes about 1 hour and covers such issues as medical history (specific diseases, pain, etc.), functional performance (strength, balance, etc.), disabilities, and previous experience with physiotherapy. Some tests for frailty are used, but not all that would be desirable. This assessment might take 1 hour, but it depends on several factors, like severe disabilities.

After this first assessment, physiotherapists classify individuals in terms of their functional ability and necessities. In this sense, they could provide individualized services on a one-to-one basis to those residents with higher needs (such as severe physical or mental impairment, or stroke). Time with each patient is highly dependent on the residents' necessities, tolerance to physiotherapy and specific workload of the nursing home. On the other hand, if possible, the residents may participate in a group-based physical exercise programme, standardized according to their physical ability. This programme would last between 45 and 60 minutes.

Physiotherapy services are usually provided only by the physiotherapist, with little involvement of nursing home staff, but they may assist with residents with special physical needs.

Working days are Monday to Friday, with no services provided on the weekend.

Physiotherapists are working in either public or private nursing homes. If they are working in public nursing homes, they are employed by one institution. Working times are usually 8.00am to 3.30pm or 8.30am to 5.00pm, with longer working times in private nursing homes, because of a longer lunch break and two more sessions in the afternoon. Salaries in public nursing homes tend to be higher than those in private ones. However, some private nursing homes might propose a higher salary to physiotherapists if they provide extra sessions to residents who pay for them.

For patients, basic physiotherapy services should be covered, irrespective of whether they are provided in public or private nursing homes. Additional sessions might be paid for by patients in the private nursing homes. This is not allowed in the public ones.

Source: Alejandro Álvarez Bustos

Access to specialized care

Regarding access to specialized care, nursing homes and regions differ widely. While some regions may benefit from geriatricians, internal medicine doctors and palliative care professionals who visit nursing homes as part of social and healthcare coordination programmes, others lack access to these specialists (Mateos-Nozal et al., 2023). The same is true for services requiring coordination with specialized care, such as facilitating access to hospital-use medication, prescribing orthopedic materials, managing health tests and coordinating with other specialists (Mateos-Nozal et al., 2023).

Needs assessment and referral

Depending on the region and staff availability, a new resident is assessed upon admission to the nursing home by nurses, nurse case managers, nurse assistants and/or NHS primary care physicians, who then develop a care plan (Regato-Pajares et al., 2023; Bernal-Delgado et al., 2024).

Access to medical care beyond the scope of nursing home care, such as specialized care, is determined by the NHS primary care physician, who attends to residents if requested by nursing home personnel, that is, nurses and nurse assistants (Bernal-Delgado et al., 2024).

3.7 Sweden

Contextual background

The Swedish health system is mainly publicly funded, primarily through regional and municipality taxation (Kirsebom et al., 2017a; Salaj, Schultz & Strang, 2024). The LTC system aims to support older people to live an independent life for as long as possible (Nilsen et al., 2018; Swedish Institute, 2024). It is based on formalized care services delivered by the country's 290 municipalities (Sköldunger, Sandman & Backman, 2020). Healthcare costs paid by older people themselves are subsidized, based on the individual's income and assets (Swedish Institute, 2024; Van Buul et al., 2020).

In general, the nursing home resident pays for the rent, as well as an income-dependent fee for basic costs in the nursing home (including daily nursing, physiotherapy, occupational therapy, etc.), which is subsidized by the municipality, whereas the region pays for all external healthcare costs (including primary care, outpatient care such as laboratory and radiology tests, medicines, emergency room visits, hospital stays if needed, etc.) (Kirsebom et al., 2017a; Salaj, Schultz & Strang, 2024). Most LTC in Sweden is provided by the public sector and financed through municipal taxes and government grants (Sköldunger, Sandman & Backman, 2020). Swedish nursing homes provide different care options, ranging from basic nursing care to specialized care for complex medical conditions (Van Buul et al., 2020). Many nursing homes have short-term units available for rehabilitation, palliative care or people waiting for a permanent bed in a nursing home, but the organization of these units varies across the 290 municipalities (Van Buul et al., 2020). As cognitive disorders and dementias are found in a majority of residents, more than half of the nursing homes have specialized dementia units and the majority of nursing homes also has beds available for people with physical diseases, as frailty and comorbidities are common as well (Lundin & Godskesen, 2021; Salaj, Schultz & Strang, 2024). In Swedish nursing homes, residents live in their own apartments designed to accommodate their specific requirements, while allowing for self-determination, autonomy and community engagement, with care provided around the clock (Nilsen et al., 2018).

Needs assessment for LTC is undertaken by the municipalities (Maino & De Tommaso, 2023). Care recipients can formally choose between private for-profit organizations and public/private non-profit providers but choice in practice depends on local conditions (Maino & De Tommaso, 2023).

Access to primary care

In Sweden medical care for those living in nursing homes is managed by a primary healthcare centre, with staff providing advice and primary care to the residents, while being employed by the municipality, with the exception of the physician, who is provided by the region on a consultancy basis (Salaj, Schultz & Strang, 2024). Nursing home residents are advised to sign an individual contract with the primary healthcare centre with which the nursing home has an agreement, in order to improve access to medical services. However, residents may also choose to stay with their previous primary healthcare centre and GP, who will then be responsible for them.

Medical care in Swedish nursing homes is mostly carried out by GPs, who usually take care of all residents and pay weekly visits to the nursing home, visiting the facility at fixed times during the week, as part of the agreement between the primary healthcare centre and the nursing home (Kirsebom et al., 2017b; Van Buul et al., 2020). Unplanned visits are avoided as much as possible to avoid conflicts with patient appointments at the primary healthcare centre, as during the rest of the week the GP usually works with outpatients (Bolmsjö et al., 2015; Kirsebom et al., 2017b). If acute consultations are needed, the GP is contacted in between weekly visits, and during out-of-office hours regionally contracted mobile physicians are available to support nursing home staff (Kirsebom et al., 2017b). Furthermore, some regions and municipalities have mobile teams of doctors who complement or even substitute for ordinary primary healthcare centres. Usually, there is more than one GP connected to a nursing home (Kirsebom et al., 2017a; Van Buul et al., 2020).

Nursing care is mainly provided by registered nurses who are on site during daytimes and who support the GP in providing medical care for nursing home residents outside the GP's visiting hours (Kirsebom et al., 2017a; Liljas et al., 2024; Lindgren et al., 2024). At night and during the weekends, care workers and assistant nurses provide 24-hour social and personal healthcare, with the opportunity to contact registered nurses and/or GPs on-call in case of an emergency (Kirsebom et al., 2017a; Backman et al., 2023; Liljas et al., 2024).

Access to allied health professionals

Access to allied health professionals depends on the municipality the nursing homes are placed in. Registered physical and occupational therapists, who work for the municipality, act also as rehabilitation consultants for nursing homes (Lindgren et al., 2024). Furthermore, speech and language therapists can be consulted as part of specialist care but are not among regular staff (Forsgren & Saldert, 2022). However, these regulations may also lead to a shortage of staff, as, for example, not all municipalities have access to dietitians (Uppsala University, 2018).

Access to specialized care

As for allied health professionals, specialized care such as psychiatry is available on a consultancy basis and may need to be provided outside the nursing home, that is, care in geriatric and psychiatric hospital departments if certain equipment is needed (Lundin & Godskesen, 2021; Liljas et al., 2024; Salaj, Schultz & Strang, 2024). Palliative care is generally available in Swedish nursing homes, and provided to terminally ill nursing home residents (Van Buul et al., 2020). Furthermore, geriatricians from geriatric clinics may sometimes provide medical and geriatric care at nursing homes in large cities and in some of the municipalities (Van Buul et al., 2020; Lundin & Godskesen, 2021). However, most healthcare in Swedish nursing homes is provided by registered nurses and GPs.

Needs assessment and referral

Medical assessment takes place before admission to a nursing home and is carried out by the municipality and in collaboration with the region, if needed. Medical assessment of a resident also takes place when their health status changes (Kirsebom et al., 2017b). The level of care provided depends on the individual's needs and can be adjusted as required (Expat Focus, 2023). In general, nurses identify patients who need a medical assessment by a physician and, in addition, physicians and nurses discuss their patients on a regular basis (Bolmsjö et al., 2015; Kirsebom et al., 2017a). Most of the physicians' work includes medical assessment of the residents and work related to assessment, such as ordering tests (such as blood samples), prescribing medicines and documenting in the healthcare record (Kirsebom et al., 2017b). Registered nurses are responsible for creating nursing care plans, describing patients' care needs, the necessary nursing interventions and the desired goals and outcomes (Kirsebom et al., 2017a).

As primary care physicians are responsible for residents' medical care, they are also responsible for advanced care planning, which should be updated at least once a year (Kirsebom et al., 2017a; Lundin & Godskesen, 2021). Individual care plans should be created during meetings with the resident, their family members, their GP and registered nurses, and should include the most important information on each resident's health status and their preferred treatment and care in case their health deteriorates, to secure a person-centred approach (Kirsebom et al., 2017a).

Contextual background

In the European countries described here, the health system is primarily publicly funded, offering a benefits package to almost the entire resident population, with financing sources including taxes and other income-dependent contributions. Co-payments for certain health services play a lesser role, and people can also opt for private insurance.

In most of the selected countries, LTC services are covered and financed similarly to health services, but co-payments may play a substantially larger role in LTC spending than in health spending. Public coverage for LTC usually only includes the cost of care and medical services in nursing homes. Expenses for rent and products and services that are not medically necessary are not covered, and nursing home residents usually have to pay a fee, depending on their pension and assets. If individuals have an income below a certain threshold or are in financial need, the government and/or social welfare covers the costs. In England around 41% of nursing home residents self-fund their care, which is a higher proportion than in the other selected countries.

In all of the countries discussed here, the need for care and the transition to a nursing home is usually assessed by an independent assessor, who then allocates the necessary care to the individual.

The proportion of nursing homes operated by non-profit, for-profit, public and private providers differs between the selected countries: while Germany and the Netherlands have many non-profit operators, most nursing homes in Norway, Poland and Sweden are public. In Spain and England the majority of nursing homes are privately owned and for-profit.

In most of the countries, nursing homes are divided into different units or wards. There is a strong focus on dementia care, given that most people admitted to nursing homes have some form of cognitive decline. Other common wards in nursing homes include those specializing in rehabilitation and palliative care. Furthermore, many nursing homes offer both short-term and long-term care.

Access to primary care

GPs tend to be the main professionals responsible for providing medical care to nursing home residents, with nursing care being provided by nursing staff or care assistants. Most GPs working in nursing homes are self-employed and community-based, and not employed by the facility. This means that they only spend a certain amount of time per week with the nursing home residents, while working the rest of the time in their own practice. Nursing home residents can also visit the GP in their practice, if their health and mobility allow.

An exception is the Netherlands, where specially trained ECPs are directly employed by nursing homes, with one full-time ECP caring for approximately 100 residents. A similar model is also used by some nursing homes in Norway that employ their own dedicated ECP who works at the facility full-time, depending on the type of nursing home and its location. They are reimbursed for this by the municipality that otherwise would employ the GPs working in nursing homes and their own practice. However, even in the Netherlands, in smaller nursing homes care may also be provided by GPs.

The ratio of physicians to residents differs between nursing homes within and between countries, as does the amount of time physicians spend with their patients in nursing homes. All nursing homes in the selected European countries provide out-of-hours and acute care.

Patients in all seven countries are in general free to choose their GP and this can affect the provision of primary care to nursing home residents. Countries differ in particular in terms of whether nursing home residents keep the GP they had prior to transitioning to the nursing home (assuming it is close to where they previously lived).

In some countries, such as Germany and Poland, it is common for residents to keep their own GP when they transition to a nursing home. In this scenario, multiple GPs tend to take care of different patients in one and the same nursing home. While in Germany nursing homes are obliged to have cooperation agreements in place with GPs, on average nursing homes are served by nine different GPs, who in turn provide services on average to 47 residents in four different nursing homes, in addition to working in their own practice.

In Norway and Sweden nursing homes have contracts with local health centres or GP practices and most nursing home residents choose to switch to these providers instead of keeping their own GP, although they retain the right to choose their GP and some do. This is also the case for many nursing homes in England and Spain.

The payment schemes for physicians providing services to nursing home residents also differ, comprising capitation and fee-for-service models, as well as fixed salaries. In Germany, for example, physicians are reimbursed through a fee-for-service model, whereas in Norway they tend to be employed by the municipalities and receive a fixed, regulated salary. In Poland and Spain both models exist and GPs providing services to nursing home residents may be either self-employed or salaried employees.

In England and Germany GPs are in principle reimbursed for their work with nursing home residents in the same way as they are paid for providing care to people living in their own homes. In England they are paid according to the standard NHS contract (mainly capitation-based), although some nursing homes offer additional financial incentives. Such special cooperation agreements that offer additional financial incentives also exist in Germany. In Sweden medical care for those living in nursing homes is managed by a primary healthcare centre, with staff providing advice and primary care to the residents, while being employed by the municipality, with the exception of the physician, who is provided by the region on a consultancy basis.

The tasks that nurses are responsible for in nursing homes depend on their level of education and nursing responsibilities differ across the selected European countries. For example, nurses in Germany or Spain are only permitted to perform basic nursing care, whereas nurses in Sweden can also provide medical care and nurses in England can be highly specialized.

Access to allied health professionals

In all seven countries many nursing homes involve physiotherapists, occupational therapists, speech and language therapists, psychologists and social workers. Allied health professionals are either directly employed by the nursing homes or work independently in private practices and provide services upon referral, either on-site or at their own practice. Whether allied health professionals are directly employed by the nursing home or work on a freelance basis depends on the country, the type and ownership of the nursing home, the region in which it is located (rural vs. urban), and its size, specialization and local policies. There are also differences between the different categories of allied health professionals.

In the Netherlands, for example, almost all nursing homes have permanently employed allied health professionals, providing care services on site, with physiotherapists, occupational therapists, speech therapists and nutritionists being the most common. This contrasts with Germany, where only about a third of nursing homes have permanently employed allied health professionals and most allied health professionals work in private practices and offer home visits to nursing homes based on a physician's prescription.

The services of allied health professionals (whether inside or outside the nursing home) tend to be publicly covered, provided they are based on a referral from a GP. In Germany, for example, care

delivered by allied health professionals is publicly covered if it is medically necessary and prescribed by a doctor, aimed at maintaining or improving the abilities of nursing home residents, or arranged by the nursing home directly. In these cases, the statutory health insurance, the LTC insurance or the nursing home pay for these services, although residents have to make co-payments (10% of the cost of therapeutic services and €10 per prescription).

Access to specialized care

Similar to allied health professionals, specialists are often brought in as consultants, rather than being part of the team working permanently in nursing homes. However, in some countries, such as Germany, nursing homes are required to have contracts in place with specialists, including dentists. Specialists usually work in private practices and may offer nursing home visits. However, given that many specialists such as dentists require specific equipment, they are much less available in nursing homes than primary care professionals. Nursing homes generally report a high demand for psychiatrists, neurologists, urologists, ophthalmologists, otolaryngologists and dentists.

Most nursing homes in the selected countries, such as Germany, the Netherlands, Sweden and England, offer palliative care. Geriatricians also support work in nursing homes, for example, in the Netherlands and Sweden. Depending on the type of nursing home and the region, multidisciplinary meetings are held regularly to discuss patients and aim to provide continuous, person-centred healthcare.

Needs assessment and referral

Upon admission to a nursing home in all of the selected countries, a geriatric assessment is carried out to develop a care plan, which is adjusted if the resident's health deteriorates. The physician is usually responsible for the CGA and care plan development, but allied health professionals (for example, physiotherapists or speech and language therapists) may also assess patients during treatment. Advanced care planning may also form part of the admission interview in countries such as the Netherlands, Norway and Sweden.

Access to GPs and other health professionals is often organized by nursing staff, who have the closest contact with the nursing home residents. In most of the selected countries, GPs act as "gatekeepers" to other medical services and their referral is needed for specialist appointments, for example in Poland and Spain. However, in some countries, such as Germany and England, nursing home residents can also choose to consult specialists independently.

Policy options for improving the provision of health services to nursing home residents

As this report illustrates, the provision of health services to nursing home residents varies substantially between and within European countries, even among countries with similar health and LTC systems (Ågotnes et al., 2019; Rostad et al., 2020; Pedersen, Skinner & Sogstad, 2024). These differences can be seen as a natural laboratory and, although evidence on their impact on patient outcomes is largely non-existent, they provide pointers to policy options and the decisions countries have to take.

Given the differences in the organization and financing of health service provision to nursing home residents, there is a case for robust quality assurance mechanisms and governance structures to make sure that health services to nursing home residents are provided equitably, are of high quality and are accessible to all who need them, irrespective of the type of nursing home they live in.

Countries need to decide on what models for providing health services to nursing home residents work best in their particular health system and LTC context, taking into account that models may need to be different in different parts of the country and reflect the size of nursing homes, as well as the surrounding healthcare infrastructure. Ideally, and where possible, primary, specialized and intermediate care should all be provided within nursing homes, so that patients do not have to change settings, avoiding acute hospital admissions, disorientation and distress (Conroy et al., 2009). The ideal is a proactive and person-centred approach that meets high standards of care (Sköldunger, Sandman & Backman, 2020; Sterke et al., 2021; Morri, 2024). This will help to enable nursing home residents to maintain their highest possible quality of life with the greatest degree of independence, autonomy, participation and human dignity (WHO, 2022a; Gesund.bund.de, 2025d).

The next set of policy questions relates to which primary care physicians should be responsible for nursing home residents, where they should be located, and how they should be paid. The Netherlands is the only country in our sample where it is common practice for specially trained ECPs to be directly employed by (larger) nursing homes, although some (larger) nursing homes in Norway follow the same model. The ECPs follow a more proactive approach, providing high-quality geriatric care and reducing the demand for specialized services in Dutch nursing homes, as they perform a wider range of services than traditional GPs, such as ophthalmological examinations (Koopmans, Pellegrom & van der Geer, 2017; Fassmer et al., 2024). The ECP model may also reduce (avoidable) hospitalizations, which are generally very low in the Netherlands compared to other countries (Koopmans, Pellegrom & van der Geer, 2017). Finally, nursing homes in the Netherlands report higher satisfaction with primary care services received by their residents compared to other countries, as well as better availability of such care, due to the direct employment of ECPs (Fassmer et al., 2024). While the use of the ECP model may come at greater initial cost, it may thus prove to be cost-effective in the long run.

Other countries opt for a model that allows patients to retain their GP when entering a nursing home. While this can in some cases support continuity of care, it also means that multiple community-based GPs might be responsible for the residents of a single nursing home, may only visit in person occasionally and may not be available out-of-hours. There is some evidence that dedicated nursing home physicians seem to enhance medical care and health outcomes for nursing home residents (Kirsebom et al., 2017b; Koopmans, Pellegrom & van der Geer, 2017). As the ECP model may not be suitable for all countries, however, an alternative is to have a cooperation agreement between the nursing home and an adjacent GP practice (as in England or Germany). Similarly, in Sweden medical care for those living in nursing homes is managed by a primary healthcare centre,

with staff providing advice and primary care to the residents. This model might be particularly attractive in countries that have GP group practices, ensuring that there is no gap in provision.

While not a major focus of this work, the role that GPs play beyond episodic curative care deserves attention. Chronic disease management, prevention and health promotion are critical components of care provided in nursing homes and can help nursing home residents to maintain their health status, preserve their mobility and prevent further deterioration (Gesund.bund.de, 2025d). Furthermore, GPs can play a role in organizing care delivery and improving care quality in nursing homes as part of multidisciplinary teams, rather than just providing patient care. It seems that the ECP model is closer to this ideal than community-based GPs, but in the latter case a clearer delineation of roles and responsibilities might help.

Countries will also need to ensure strong mechanisms for the integration of care and the transition between care settings. Improved coordination between different care providers (that is, primary care, specialized care and hospital care) has the potential to reduce avoidable hospitalizations, while improving discharge processes and the development of more person-centred services (Kirsebom et al., 2017b; Allers et al., 2020; Schröder et al., 2020).

The gatekeeping function of GPs and their role in coordinating patient care are particularly relevant in this context. In some of the countries, such as England and the Netherlands, GPs have a strong gatekeeping role. Specialized health services require referral, and GPs tend to offer a wider range of health services and play a stronger role in coordinating patient care. In contrast, gatekeeping is underdeveloped in countries such as Germany, where patients can access specialists without referral and care coordination may be weaker.

Models for the provision of services by allied health professionals to nursing home residents differ across countries. They can be either directly employed by the nursing home or work on a freelance basis and we could not identify evidence on the effectiveness or cost-effectiveness of the different models. Much will depend on the specific health system and LTC context and the location and size of the nursing home.

Finally, robust assessment procedures, such as CGAs, can help to ensure the provision of appropriate care (Conroy et al., 2009). As the literature suggests, allied health professionals hereby play a crucial role in helping to recognize specific medical needs (Fleurke, Voskuil & Beneken genaamd Kolmer, 2020).

It is also important to note that many of the challenges highlighted in this report do not only pertain to nursing home residents, but relate to wider issues for health and LTC systems. These include the questions of how well developed the primary care system is, whether it fulfils a gatekeeping function and how it contributes to the overall coordination of care. Access to publicly covered allied health professionals is another challenge for health systems in general and does not only affect nursing home residents. One of the questions policy-makers will need to address is whether access to health services for nursing home residents should follow the same pathways and referral mechanisms as for people outside nursing homes or whether expedited access would be more appropriate.

This report has explored the provision of health services to nursing home residents in seven European countries and policy options for making improvements. It found substantial variation in the organization and funding of LTC systems, the ownership and organization of nursing homes, and the provision of primary and specialist health services to nursing home residents. The differences between and within countries provide some illustrations of where things could be organized differently, although evidence on impact is limited and difficult to ascertain.

What is clear is that the countries face very similar challenges and that these are growing. LTC systems tend to be strained and underfunded, and workforce shortages are common. Frail older people in nursing homes are especially vulnerable and it is easy to overlook their needs for high-quality and dignified health and LTC.

Yet, as this report has shown, there are a number of actions countries can consider to improve the provision of health services to nursing home residents. They include decisions on how to ensure access to primary care, allied health professionals and specialized care. Many of these decisions have so far been taken rather implicitly and by default. Much could be gained by opening them up to public consultation and explicit decision-making and this report hopes to provide pointers to underpin these efforts.

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